WOUND CARE DICTATION GUIDE

Innovative Wound Consultants

Wound Exam / Treatment

Patient Name: _____
Date-of Service: _____
Provider: _____
Place of Service: patient's home ☐ assisted living facility ☐, nursing home ☐, SNF physician office ☐.

Potential Barriers to Wound Healing

Diabetes

- Is the patient diabetic? Yes ☐ No ☐
  - Yes →
    - ☐ Type 1 or ☐ Type 2
    - A1c level: _____
      - If the A1C is above 8.0 treatment, determine if the wound is healing effectively.
    - Average daily glucose level? _____
    - Name of PCP / Specialist currently managing patient's co-morbidities: _____
  - No → move to next barrier

Venous/Arterial Insufficiency

- Does the patient have sufficient vascularization? Yes ☐ No ☐
  - Dictate venous/arterial diagnostic performed and results:
    - ABI : _____
    - Pulse Volume Recording: _____
    - Duplex scan: _____
    - Doppler Ultrasound: _____
    - Other Vascular / Arterial Assessment: _____
    - Is vascular referral needed?
      - Yes ☐ No ☐

Nutrition & Drug/Tobacco/Alcohol Habits

- Is the patient receiving proper nutrition? Yes ☐ / No ☐
  - Have you ordered Labs (CBC w/diff w/PLT, CMP, A1C)? Yes ☐ / No ☐
- Is the patient a smoker? Yes ☐ / No ☐
  - Have you consulted the patient on the negative systemic effects of smoking & wound healing - and encouraged the patient to stop smoking during treatment? Yes ☐ / No ☐
  - Describe Smoking Cessation Plan: _____
- Is the patient an alcoholic? Yes ☐ / No ☐
  - Have you consulted the patient on the damaging effects of alcohol to their body and recommended that they reduce/eliminate their consumption?
  - Yes ☐ / No ☐
    - Recommended to order Labs to determine hydration, red/white blood cell count, nutrition

Offloading:

Braden Scale

- Sensory Perception: ____
- Moisture: ____
- Activity: ____
- Mobility: ____
- Nutrition: ____
- Friction and Shear: ____

Scoring Key

- Total = _____
  - Severe Risk < 9
  - High Risk = 10-12
  - Moderate Risk = 13-14
  - Mild Risk = 15-18

Norton Scale

- Physical Condition: ☐ 1 ☐ 2 ☐ 3 ☐ 4
- Mental Condition: ☐ 1 ☐ 2 ☐ 3 ☐ 4
- Activity: ☐ 1 ☐ 2 ☐ 3 ☐ 4
- Mobility: ☐ 1 ☐ 2 ☐ 3 ☐ 4
- Incontinence: ☐ 1 ☐ 2 ☐ 3 ☐ 4

Scoring Key

- Total = _____
  - Greater than 18 [Low Risk]
  - Between 18 and 14 [Medium Risk]
  - Between 14 and 10 [High Risk]
  - Lesser than 10 [Very High Risk

- Have measures been taken to offload the wound? Yes ☐ No ☐
- Yes → What measures have been taken? For how long? And what were the results?
  - Pressure relieving / Pressure redistribution mattress:
  - ROHO cushion: Yes ☐ No ☐
  - Offloading Boots: Yes ☐ No ☐
  - _____
- No → Recommend/Order offloading measures or explain why offloading is not an option.

Other Barriers:

- Does the patient have a history of previous ulcerations, infection, osteomyelitis, or amputation? Yes ☐ / No ☐
- If Yes, please elaborate. _____

Past Wound Care Treatment(s): Outline all relevant past wound care treatments, and if they have failed (50% or less reduction in 30 days), document why you believe these therapies were unsuccessful and the percentage increase or decrease in wound size over the past 30 days.

Additional Factors to Consider:

- Failed conservative care is generally considered to be less than 50% reduction in wound size over a 30-day period, with the appropriate use of advanced wound care products, management of underlying co-morbidities, and SOC for moist wound healing.

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Pre-Debridement Wound Characteristics:

- Location: _____
- Location Comment: _____
- Wound Duration: _____ years, _____ months
- Wound Type:
  - Diabetic ☐ Venous ☐ Arterial ☐ Pressure ☐ Surgical ☐ Trauma ☐ Other ☐
- Wound Stage / Grade:
  - Wagner grade (Diabetic): Grade 0 ☐ Grade 1 ☐ Grade 2 ☐ Grade 3 ☐ Other Ulcer: Stage 1 ☐ Stage 2 ☐ Stage 3 ☐ Stage 4 ☐ MDRPI ☐ DTI ☐

- For a VLU
  - Present for 3+ months? Yes ☐ / No ☐
  - Compliant with compression therapy? Yes ☐ / No ☐
  - Compression therapy duration: _____
  - Type of compression? _____
- Wound thickness: Full thickness ☐ Partial thickness ☐
- Size in cm: _____ cm length x _____ cm width x _____ cm depth, measuring from head-to-toe
- Previous size: _____ cm length x _____ cm width x _____ cm depth
- Undermining or Tunneling: U/T _____ cm@ _____ o'clock
- Undermining ☐ Tunneling ☐ Neither ☐

- Wound Bed:
  - Granular Tissue %: _____ %
  - Fibrotic Tissue %: _____ %
  - Slough Tissue %: _____ %
  - Eschar %: _____ %
- Wound Color: Pink ☐ Red ☐ Yellow ☐ Black ☐
- Visible Structures:
  - Tendon ☐ Bone ☐ Muscle ☐ Capsule ☐ Ligament ☐ None ☐ Hardware ☐
- Wound Edges: Attached ☐ Unattached ☐ Calloused ☐ Rolled or Epibole ☐
- Periwound tissue: Healthy ☐ Macerated ☐ Inflamed ☐ calloused ☐
- Exudate Amount: Minimal ☐ Moderate ☐ Heavy ☐ None ☐
- Exudate Type: Serous ☐ Serosanguinous ☐ Purulent ☐ Bloody ☐ None ☐ Other ☐
- Exudate Color: Clear ☐ Pink ☐ Yellow ☐ Cream ☐ Green ☐ None ☐
- Signs / Symptoms for Infection: Yes ☐ No ☐
- Yes ->
  - Describe (osteo, MRSA, etc.) _____ Acute ☐ Chronic ☐
    - Intervention: Cx ☐ PCR ☐ Biopsy ☐ Other ☐
    - Treatment Plan:
- Pain Scale [0-10]:
    - Describe location, type, affect on debridement, compression, etc.
    - Rx Medications:
- Wound progression:
- _____ % change in Surface Area; _____ % change in Volume

Post-Debridement Wound Characteristics:

- Post-Debridement Measurements:
- _____ cm length x
- _____ cm width x
- _____ cm depth
- Debridement Type: None ☐ Autolytic ☐ Enzymatic ☐ Mechanical ☐ Surgical ☐ Sharp ☐
- Non-Contact MIST ☐ Other ☐
- Tissue Removed:
- Wound Bed:
- Post-Debridement Granular Tissue %: _____ % Post
- Debridement Fibrotic Tissue %: _____ % Post
- Debridement Slough Tissue %: _____ % Post
- Debridement Eschar %: _____ %
- Instrument(s) Used:
- Blood Loss:
- Primary Dressing(s): _____
- Secondary Dressing(s): _____
  - For Skin Subs:
    - Product Name & Size _____
    - HCPCS:
    - Amount Used: _____ cm; Amount Wasted: _____ cm
    - Tissue ID: _____
    - Expiration Date: _____

Treatment Plan

- Treatment Plan Goals: _____
- Dressing Change Protocol: _____
- Advanced Modalities: _____
- Specialists Referrals: _____
- Medications: _____
- Additional Info: _____

Medical Necessity Statement:

The patient has undergone greater than 4 weeks of conservative wound care treatments without resolution. We discussed benefits, risks, and complications of skin substitutes with patient. [Insert Product Name] allograft will be used as a wound cover to maintain a moist wound environment and to protect from the outside environment in an attempt to promote wound progress and prevent (re-)hospitalization. Graft #1 was placed today with patient consent. We will continue to evaluate the current care plane, and consider continued use of skin substitutes as deemed necessary.

_____

*** *Repeat for any additional wounds* ***

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